Separate the three policy clocks

Companion: Before You Buy AI for Prior Authorization, Check the 2027 Rules | November 5, 2026

1 / Current CMS implementation overview

Use to verify current readiness guidance and links. Hospital/CAH electronic-prior-authorization reporting: optional bonus in 2027, mandatory in 2028. Discuss native integration, modules, timelines and testing with EHR partners.

Open current CMS readiness guidance

2 / Finalized non-drug framework: CMS-0057-F

API dates generally begin January 1, 2027; exact timing varies by payer type. Scope includes affected MA, Medicaid/CHIP and exchange QHP payers, not every commercial product. Decision-time provisions generally start in 2026; exchange QHP issuers are excluded from those timeframes.

Open CMS-0057-F final-rule fact sheet

3 / Hospital reporting: revised timeline

The FY 2027 hospital payment final rule is the current source for hospital/CAH reporting timing. The older 2024 fact sheet's hospital timeline should not anchor a current implementation brief.

Open FY 2027 hospital final-rule summary

Source check: November 3, 2026. Applicability depends on the program, payer product, benefit and service. Your compliance owner should confirm local obligations and subsequent changes.

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What the evidence does not establish

Direct sources support the policy distinctions. The buying method is an ALHQ editorial proposal.

4 / Proposed drug provisions: CMS-0062-P

CMS identifies this as a proposal in the sources checked. Provisions would begin October 1, 2027. Do not present proposed timing as settled or assume the finalized non-drug framework covers every medication workflow.

Open CMS drug prior-authorization proposal

Keep policy readiness separate from AI value

These sources do not validate a product, prove savings, promise immediate approval or establish local interoperability. An API connection and a generated clinical summary are different capabilities.

Five original ALHQ procurement records

Coverage, transaction, evidence, work and economics translate the approved newsletter into a local buying discussion. The workbook is not a CMS template, validated score, universal pass threshold or legal opinion.

Controlled comparison, then a bounded decision

Use authorized test environments and matched synthetic cases. Do not duplicate a live submission. Prespecify criteria, retain failures and unresolved episodes, trace clinical assertions and name fallback owners. No patient details belong in the workbook.

State the financial mechanism

Measure active work across the full eligible workload. Separate released capacity from verified cash benefit, and subtract incremental costs once. No vendor outcome, clinical benefit or return on investment has been observed by this tool.

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